

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Full Study Title:	A Phase III, Randomized, Double-blind, Multicenter, Global Study of Rilvegostomig or Pembrolizumab in Combination With Platinum-based Chemotherapy for the First-line Treatment of Patients With Metastatic Squamous Non-small Cell Lung Cancer Whose Tumors Express PD-L1 (ARTEMIDE-Lung02)
Short Title:	A Global Phase III Study of Rilvegostomig or Pembrolizumab Plus Chemotherapy for First-Line Treatment of Metastatic Squamous Non-small Cell Lung Cancer (NSCLC)
Protocol Number:	66339227367613570
NCT Number:	NCT06692738
Sponsor Name:	AstraZeneca
Investigational Product:	carboplatin
Phase:	PHASE3
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Overall survival (OS): OS is defined as the time from randomization until the date of death due to any cause. 2. Progression-free survival (PFS): PFS is defined as the time from randomization until radiological progression per Response Evaluation Criteria in Solid Tumors, Version 1.1 (RECIST 1.1) or death due to any cause (in the absence of progression).
Secondary Obj.:	1. Landmark overall survival (OS) rates 2. Landmark progression-free survival (PFS) rates 3. Time to second progression or death (PFS2)

2.2 Background & Rationale

The purpose of ARTEMIDE-Lung02 is to assess the efficacy and safety of rilvegostomig in combination with platinum-based chemotherapy for the first-line (1L) treatment of patients with metastatic squamous non-small cell lung cancer (mNSCLC) whose tumors express programmed death-ligand 1 (PD-L1).

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE3, Status: RECRUITING
Target N:	880

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * Histologically or cytologically documented squamous NSCLC.
- * Stage IV mNSCLC (based on the American Joint Committee on Cancer Edition 8) not amenable to curative treatment.

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- * Absence of documented tumor genomic mutation results from tests conducted as part of standard local practice in any actionable driver oncogenes for which there are locally approved targeted 1L therapies.
- * Provision of acceptable tumor sample to confirm tumor PD-L1 expression TC ≥ 1%.
- * At least one lesion not previously irradiated that qualifies as a RECIST 1.1 TL at baseline and can be accurately measured at baseline as ≥ 10 mm in the longest diameter (except lymph nodes, which must have short axis ≥ 15 mm) with CT or MRI and is suitable for accurate repeated measurements.
- * Adequate organ and bone marrow function.

4.2 Exclusion Criteria

- * Presence of small cell and neuroendocrine histology components.
- * Brain metastases unless asymptomatic, stable, and not requiring steroids or anticonvulsants for at least 7 days prior to randomization. A minimum of 2 weeks must have elapsed between the end of local therapy (brain radiotherapy or surgery) and randomization. Participants must have recovered from the acute toxic effect of radiotherapy (eg, dizziness and signs of increased intracranial pressure) or surgery prior to randomization.
- * Any prior systemic therapy received for advanced or mNSCLC.
- * Any prior treatment with an anti-PD-1 or anti-PD-L1 agent.
- * Any prior exposure to an anti-TIGIT therapy or any other anticancer therapy targeting immune-regulatory receptors or mechanisms.
- * History of another primary malignancy except for malignancy treated with curative intent with no known active disease ≥ 2 years before the first dose of study intervention and of low potential risk for recurrence.
- * Active or prior documented autoimmune or inflammatory disorders requiring chronic treatment with steroids or other immunosuppressive treatment.
- * Active primary immunodeficiency/active infectious disease(s).
- * Active tuberculosis infection.

11. Study Identifiers & Governance

Primary ID: 66339227367613570

Registry Number: NCT06692738

Sponsor: AstraZeneca

Study Title: A Global Phase III Study of Rilvegostomig or Pembrolizumab Plus Chemotherapy for First-Line Treatment of Metastatic Squamous Non-small Cell Lung Cancer (NSCLC)

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____